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For submit in Malaysia

Artwork of insert “GOFEN 400” code *RI-I030-H14-00-02*

Size : 248 x 186 mm (*5x10’S with catchcover*)

31 mm

186 mm

(Back)

Probenecid and sulfinpyrazone:	Medicinal products that contains probenecid or sulfinpyrazone may delay the excretion of ibuprofen.
Diuretics, ACE inhibitors, beta-receptor-blockers and angiotensin-II antagonists:	NSAIDs may reduce the effect of diuretics and other antihypertensive medicinal products. In some patients with compromised renal function (e.g. dehydrated patients or elderly patients with compromised renal function) the co-administration of an ACE inhibitor, beta-receptor-blockers or angiotensin-II antagonists and agents that inhibit cyclo- oxygenase may result in further deterioration of renal function, including possible acute renal failure, which is usually reversible. Therefore, the combination should be administered with caution, especially in the elderly. Patients should be adequately hydrated and considered should be given to monitoring of renal function after initiation of concomitant therapy, and periodically thereafter.
Potassium sparing diuretics:	The concomitant administration of GOFEN and potassium-sparing diuretics may lead to hyperkalaemia (check of serum potassium is recommended).
Methotrexate:	The administration of GOFEN within 24 hours before or after administration of methotrexate may lead to elevated concentrations of methotrexate and an increase in its toxic effect.
Ciclosporin:	The risk of a kidney-damaging effect due to ciclosporin is increased through the concomitant administration of certain nonsteroidal anti-inflammatory drugs. This effect also cannot be ruled out for a combination of ciclosporin with ibuprofen.
Tacrolimus:	The risk of nephrotoxicity is increased if the two medicinal products are administered concomitantly.
Zidovudine:	There is evidence of an increased risk of haemarthroses and haematoma in HIV (+) haemophiliacs receiving concurrent treatment with zidovudine and ibuprofen. Increased risk of haematological toxicity when NSAIDs are given with zidovudine.
Sulphonylureas:	Clinical investigations have shown interactions between nonsteroidal anti-inflammatory drugs and antidiabetics (sulphonylureas). Although interactions between ibuprofen and sulphonylureas have not been described to date, a check of blood-glucose values is recommended as a precaution on concomitant intake.
Quinolone antibiotics:	Animal data indicate that NSAIDs can increase the risk of convulsions associated with quinolone antibiotics. Patients taking NSAIDs and quinolones may have an increased risk of developing convulsions.
Mifepristone:	NSAIDs should not be used for 8-12 days after mifepristone administration as NSAIDs can reduce the effect of mifepristone.
CYP2C9 inhibitors:	Concomitant administration of ibuprofen with CYP2C9 inhibitors may increase the exposure to ibuprofen (CYP2C9 substrate). In a study with voriconazole and fluconazole (CYP2C9 inhibitors), an increased S(+)-ibuprofen exposure by approximately 80 to 100 % has been shown. Reduction of the ibuprofen dose should be considered when potent CYP2C9 inhibitors are administered concomitantly, particularly when high-dose ibuprofen is administered with either voriconazole and fluconazole.

Pregnancy and Lactation:
Pregnancy
Inhibition of prostaglandin synthesis may adversely affect the pregnancy and/or the embryo/fetal development. Data from epidemiological studies raise concern about an increased risk of miscarriage and of cardiac malformation and gastroschisis after use of a prostaglandin synthesis inhibitor in early pregnancy. The absolute risk for cardiovascular malformation was increased from less than 1%, up to approximately 1.5%. The risk is believed to increase with dose and duration of therapy.

In animals, administration of a prostaglandin synthesis inhibitor has been shown to result in increased pre- and post-implantation loss and embryo-fetal lethality. In addition, increased incidences of various malformations, including cardiovascular, have been reported in animals given a prostaglandin synthesis inhibitor during the organogenetic period.

During the first and second trimester of pregnancy, ibuprofen should not be given unless clearly necessary. If ibuprofen is used by a woman attempting to conceive, or during the first and second trimester of pregnancy, the dose should be kept as low and duration of treatment as short as possible.

During the third trimester of pregnancy, all prostaglandin synthesis inhibitors may expose:

- the foetus to:
 - cardiopulmonary toxicity (with premature closure of the ductus arteriosus and pulmonary hypertension);
 - renal dysfunction, which may progress to renal failure with oligo-hydramnios;
- the mother and the neonate, at the end of pregnancy, to:
 - possible prolongation of bleeding time, an anti-aggregating effect which may occur even at very low doses;
 - inhibition of uterine contractions resulting in delayed or prolonged labour. Consequently, ibuprofen is contraindicated during the third trimester of pregnancy.

Use of NSAIDs at about 20 weeks gestation or later in pregnancy may cause fetal renal dysfunction leading to oligohydramnios and in some cases, neonatal renal impairment. These adverse outcomes are seen, on average, after days to weeks of treatment, although oligohydramnios has been infrequently reported as soon as 48 hours after NSAID initiation. Oligohydramnios is often, but not always, reversible with treatment discontinuation.

Lactation
Ibuprofen and its metabolites can pass in low concentrations into the breast milk. No harmful effects to infants are known to date, so for short-term treatment with the recommended dose for pain and fever interruption of breast-feeding would generally not be necessary.

Side Effects:
The list of the following undesirable effects comprises all undesirable effects that have become known under treatment with ibuprofen, also those under high-dose long-term therapy in rheumatism patients. The stated frequencies, which extend beyond very rare reports, refer to the short-term use of daily doses up to a maximum of 1200 mg ibuprofen for oral dosage forms and a maximum of 1800 mg for suppositories.

With the following adverse drug reactions, it must be accounted for that they are predominantly dose-dependent and vary interindividually.

The most commonly observed adverse reactions are gastrointestinal in nature: Peptic ulcers, perforation GI bleeding, sometimes fatal, particularly in the elderly may occur (see section Warnings and Precautions). Nausea, vomiting, diarrhoea, flatulence, constipation, dyspepsia, abdominal pain, melena, haematemesis, ulcerative stomatitis, exacerbation of colitis and Crohn's disease (see section Warnings and Precautions) have been reported following administration. Less frequently, gastritis has been observed. Particularly the risk of gastrointestinal bleeding occurring is dependent on the dose range and the duration of use.

Oedema, hypertension and cardiac failure have been reported in association with NSAID treatment.

Clinical trial and epidemiological data suggest that use of ibuprofen, particularly at high dose (2400 mg daily), and in long term treatment may be associated with a small increased risk of arterial thrombotic events (for example myocardial infarction or stroke) (see section Warnings and Precautions). Please note that within each frequency grouping, undesirable effects are presented in order of decreasing seriousness.

Very common (≥1/10), Common (≥1/100 to <1/10), Uncommon (≥1/1,000 to <1/100), Rare (≥1/10,000 to <1/1,000), Very rare (≥1/100,000 to <1/10,000).

Hypersensitivity reactions have been reported and these may consist of:

- non-specific allergic reactions and anaphylaxis
- respiratory tract reactivity, e.g. asthma, aggravated asthma, bronchospasm, dyspnoea
- various skin reactions, e.g. pruritus, urticaria, angioedema and more rarely exfoliative and bullous dermatoses (including epidermal necrolysis and erythema multiforme)

The patient is to be instructed to inform a doctor at once and to stop taking GOFEN if they experience any of the above.

Artwork Check / Layout Check					
PC TEAM		PRODUCTION		RA	
Provide by : <i>Supawan</i> Date : <i>07-Dec-23</i>	PC Check ☐ Pass : ☐ Not Pass :	Production Check 1 :	☐ Pass : ☐ Not pass :	Need to send for customer approval ☐ Yes, please send ☐ Not require	
- Revise as comment for submit.		Production Check 2 :	☐ Pass : ☐ Not pass :		
- Revise as comment : 08-Dec-23, 26-Mar-24		Production Department Head :	☐ Pass : ☐ Not pass :		
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****Specification****

Offset Printing Paper
060 gsm

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